

University Medical Center - Leukemia & BMT Clinic
Outpatient Progress Note

Patient: Thompson, Matthew
MRN: AML017
DOB: 08-Sep-1954
Date of Visit: 27-Mar-2024

SUBJECTIVE:

Mr. Thompson is a 69-year-old male with a history of intermediate-risk AML, diagnosed in early December 2021, status-post induction with CPX-351 started on 28-Dec-2021 followed by a matched unrelated donor (MUD) allogeneic stem cell transplant (SCT) in May 2022. He is now nearly 2 years post-transplant and presents for his routine follow-up. He reports feeling generally well. He notes persistent mild dry eyes and dry mouth, consistent with his known chronic Graft-versus-Host Disease (GvHD) of the eyes and oral mucosa. He denies any skin rashes, diarrhea, or shortness of breath. His energy levels are good, allowing him to garden for 1-2 hours daily. He denies any fevers, chills, new bruising, or bleeding.

OBJECTIVE:

Vitals: T 36.8°C, HR 74, BP 132/78 mmHg, RR 16, SpO2 98% on RA.

Weight: 84.5 kg (stable).

General: Well-appearing male, in no acute distress.

HEENT: Sclerae anicteric. Mild conjunctival injection noted. Oral mucosa is slightly dry but without ulcerations.

Lungs: Clear to auscultation bilaterally.

CV: Regular rate and rhythm, no murmurs.

Skin: No new rashes or lesions. Faint, stable macular rash on forearms consistent with chronic GvHD.

Ext: No edema.

Labs (27-Mar-2024):

- **CBC:** WBC 6.2 K/uL, Hgb 13.8 g/dL, Plt 225 K/uL, ANC 4.1 K/uL.
- **CMP:** All within normal limits. Cr 0.9 mg/dL. LFTs are stable.
- **Tacrolimus Level:** 6.1 ng/mL (Therapeutic range 5-10 ng/mL).
- **CMV PCR:** Not detected.
- **Chimerism (from 3 months ago):** >99% donor CD3/CD33.

ASSESSMENT & PLAN:

Mr. Thompson is a 69-year-old male with intermediate-risk AML who remains in a durable complete remission, now 27 months from the completion of his first-line therapy and 22 months post-allogeneic SCT. His clinical status is excellent.

1. **AML Status:** Remains in stringent complete remission. Surveillance bone marrow biopsy performed 3 months ago was negative for disease by morphology, flow cytometry, and MRD testing. Will continue routine monitoring. No evidence of relapse.
2. **Chronic GvHD:** Stable, mild oral and ocular GvHD. This is well-controlled on his current regimen. We will continue the very slow taper of his immunosuppression.
 - **Plan:** Decrease Tacrolimus to 1 mg PO daily. Recheck level in 2 weeks. Continue lubricating eye drops and artificial saliva as needed.
3. **Prophylaxis:** Given his ongoing immunosuppression, he will continue antimicrobial prophylaxis.
 - **Plan:** Continue Acyclovir and Dapsone (for PCP prophylaxis).
4. **Health Maintenance:** Up to date on post-transplant vaccinations. Counseled on sun protection.
5. **Comorbidities:**
 - **Hyperlipidemia:** Continue Atorvastatin 20 mg daily.
 - **Osteoarthritis:** Manages with OTC acetaminophen as needed.

Follow-up: Return to clinic in 3 months for follow-up. Will require annual bone marrow biopsy for surveillance. Continue to hold a positive outlook.

Dr. Benjamin Carter, MD
Hematology/BMT Service